

# Synthetic Patient Chart - Meets Criteria (Great Saphenous Vein)

## Patient Demographics

Name: Jane R. Whitfield (synthetic). DOB: 1968-03-11 (age 58). Sex: Female. MRN: SYN-00042. Encounter date: 2026-06-02. Referring provider: Dr. A. Cascio, Vascular Surgery.

## History

58-year-old female presents with a several-year history of symptomatic right lower extremity varicose veins. She reports persistent aching, swelling, and burning of the right leg that worsens by end of day and significantly interferes with prolonged standing at her job as a retail manager. No history of deep venous thrombosis. No known bleeding disorder. No prior venous procedures on the right leg.

## Physical Exam

Right lower extremity: visible tortuous varicosities along the medial thigh and calf, largest cluster measuring approximately 5 mm in diameter. Skin changes consistent with CEAP class C4a (pigmentation and mild eczematous changes) over the medial malleolus. No active or healed ulceration. Mild pitting edema at the ankle. Distal pulses intact. Left lower extremity unremarkable.

## Duplex Ultrasound Report

Venous duplex ultrasound of the right lower extremity performed 2026-05-20. Great saphenous vein (GSV) demonstrates reflux with a reflux time of 4.2 seconds (abnormal,  $>0.5$  s), maximal GSV diameter 6.5 mm at the mid-thigh. Deep venous system patent and competent throughout, no evidence of deep venous thrombosis. No perforator or small saphenous vein reflux identified.

## Prior Treatment

Patient has worn 20-30 mmHg graduated compression stockings daily for the past 4 months as directed, in addition to leg elevation. Despite consistent adherence, her aching, swelling, and burning symptoms have not improved and continue to limit her ability to stand for extended periods at work.

## Plan/Order

Discussed treatment options including continued conservative management versus endovenous ablation. Given demonstrated GSV reflux, CEAP C4a disease, varicosities  $\geq 3$  mm, and failure of at least 3 months of compression therapy to relieve symptoms interfering with activities of daily living, the patient elects to proceed with intervention. Plan: endovenous radiofrequency ablation of the right great saphenous vein (CPT 36475). Risks, benefits, and alternatives discussed; patient consents.